

"I'm from Marshalltown, Iowa. That's 60 miles northwest, northeast of Des Moines, Iowa. And I'm married at the present time. I'm 36 years old. My wife is 35. She lives in Garwin, Iowa. That's 15 miles southeast of Marshalltown, Iowa. I'm getting a divorce at the present time. And I am at present in a mental institution in Iowa City, Iowa, which is 100 miles southeast of Marshalltown, Iowa."

Perseveration is most commonly seen in Organic Mental Disorders, Schizophrenia, and other psychotic disorders.

PERSONALITY. Deeply ingrained patterns of behavior, which include the way one relates to, perceives, and thinks about the environment and oneself. Personality *traits* are prominent aspects of personality, and do not imply pathology. Personality *disorder* implies inflexible and maladaptive patterns of sufficient severity to cause either significant impairment in adaptive functioning or subjective distress.

PHOBIA. A persistent, irrational fear of a specific object, activity, or situation that results in a compelling desire to avoid the dreaded object, activity, or situation (the phobic stimulus). More commonly, the individual does actually avoid the feared situation or object, though he or she recognizes that the fear is unreasonable and unwarranted by the actual dangerousness of the object, activity, or situation. Some individuals with a phobia claim that their avoidance is rational because they anticipate overwhelming anxiety or some other strong emotion that is out of their control; they do not claim, however, that their anxiety is rationally justified.

POVERTY OF CONTENT OF SPEECH. Speech that is adequate in amount but conveys little information because of vagueness, empty repetitions, or use of stereotyped or obscure phrases. The interviewer may observe that the individual has spoken at some length but has not given adequate information to answer a question. Alternatively, the individual may provide enough information to answer the question, but require many words to do so, so that his or her lengthy reply can be summarized in a sentence or two. The term poverty of content of speech is generally not used when the speech is, for the most part, not understandable (incoherence).

Example: Interviewer: "OK. Why is it, do you think, that people believe in God?" Patient: "Well, first of all because, He is the person that, is their personal savior. He walks with me and talks with me. And uh, the understanding that I have, a lot of peoples, they don't really know their own personal self. Because they ain't, they all, just don't know their own personal self. They don't, know that He uh, seemed like to me, a lot of em don't understand that He walks and talks with them. And uh, show 'em their way to go. I understand also that, every man and every lady, is just not pointed in the same direction. Some are pointed different. They go in their different ways. The way that Jesus Christ wanted 'em to go. Myself, I am pointed in the ways of uh, knowing right from wrong, and doing it. I can't do any more, or not less, than that."

POVERTY OF SPEECH. Restriction in the amount of speech, so that spontaneous speech and replies to questions are brief and unelaborated. When the condition is marked, replies may be monosyllabic, and some questions may be unanswered.

Poverty of speech occurs frequently in Schizophrenia, major depressive episodes, and Organic Mental Disorders, such as Dementia.

PRESSURE OF SPEECH. Speech that is increased in amount, accelerated, and difficult or impossible to interrupt. Usually it is also loud and emphatic. Frequently, the individual talks without any social stimulation and may continue to talk even though no one is listening.

Pressure of speech is most often seen in manic episodes, but may also occur in some cases of Organic Mental Disorders, Major Depression with psychomotor agitation, Schizophrenia, other psychotic disorders, and, occasionally, acute reactions to stress.

PRODROMAL. Early signs or symptoms of a disorder.

PSEUDODEMENTIA. Clinical features resembling a Dementia that are not due to organic brain dysfunction or disease. Pseudodementia may occur in a major depressive episode or may be seen in Factitious Disorder with Psychological Symptoms.

PSYCHOMOTOR AGITATION. Excessive motor activity associated with a feeling of inner tension; the activity is usually nonproductive and repetitious. When the agitation is severe, it may be accompanied by shouting or loud complaining. The term should be used in a technical sense to refer only to states of tension or restlessness that are accompanied by observable excessive motor activity. Examples: Inability to sit still, pacing, wringing of hands, pulling at clothes.

PSYCHOMOTOR RETARDATION. Visible generalized slowing down of physical reactions, movements, and speech.

PSYCHOTIC. A term indicating gross impairment in reality testing. It may be used to describe the behavior of an individual at a given time, or a mental disorder in which at some time during its course all individuals with the disorder have grossly impaired reality testing. When there is gross impairment in reality testing, the individual incorrectly evaluates the accuracy of his or her perceptions and thoughts and makes incorrect inferences about external reality, even in the face of contrary evidence. The term psychotic does not apply to minor distortions of reality that involve matters of relative judgment. For example, a depressed person who underestimated his achievements would not be described as psychotic, whereas one who believed he had caused a natural catastrophe would be so described.

Direct evidence of psychotic behavior is the presence of either delusions or hallucinations without insight into their pathological nature. The term psychotic is sometimes appropriate when an individual's behavior is so grossly disorga-

nized that a reasonable inference can be made that reality testing is disturbed. Examples include markedly incoherent speech without apparent awareness by the person that the speech is not understandable, and the agitated, inattentive, and disoriented behavior seen in Alcohol Withdrawal Delirium.

In DSM-II the term psychotic was applied to individuals whose "mental functioning [was] sufficiently impaired to interfere grossly with their capacity to meet the ordinary demands of life," whether or not there was impaired reality testing. This definition of psychotic did not conform to common usage, which generally limited use of the term to impairment in reality testing, as does the DSM-III definition. As a result, the value of the term for communication was diminished, since it was then unclear whether or not an individual described as being psychotic had gross impairment in reality testing. It should also be noted that an individual with a nonpsychotic mental disorder may exhibit psychotic behavior, though rarely. For example, an individual with Obsessive Compulsive Disorder may at times come to believe in the reality of the danger of being contaminated by shaking hands with strangers.

In DSM-III the psychotic disorders include Pervasive Developmental Disorders, Schizophrenic and Paranoid Disorders, Psychotic Disorders Not Elsewhere Classified, some Organic Mental Disorders, and some Affective Disorders.

RESIDUAL. The phase of an illness that occurs after remission of the florid symptoms or the full syndrome. Examples: The residual states of Infantile Autism, Attention Deficit Disorder, and Schizophrenia.

SIGN. An objective manifestation of a pathological condition. Signs are observed by the examiner rather than reported by the individual.

SYMPTOM. A manifestation of a pathological condition. Although in some uses of the term it is limited to subjective complaints, in common use "symptom" includes objective signs of pathological conditions as well.

SYNDROME. A grouping of symptoms that occur together and that constitute a recognizable condition. The term "syndrome" is less specific than "disorder" or "disease." The term "disease" generally implies a specific etiology or pathophysiological process. In DSM-III most of the disorders are, in fact, syndromes.

Annotated Comparative Listing of DSM-II and DSM-III*

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Appendix C:

Annotated Comparative Listing of DSM-II and DSM-III*

This section lists all of the specific categories included in the previous manual (DSM-II) and the specific DSM-III categories that are equivalent to or subsumed by them. Because of the greater precision with which the DSM-III categories are described and because the diagnostic concepts have often been modified, the degree of equivalence varies. (For example, in DSM-III the category of Schizophrenia is more restrictive than the DSM-II category.) Whenever a category in one manual corresponds to several categories in the other, the latter categories are enclosed by one brace. In some instances, several categories from one manual are equivalent to several categories from the other manual. In such cases, a double brace is used.

This text should not be used as a conversion between DSM-II and ICD-9-CM. In a number of instances, the conceptual reclassification of disorders is unique to DSM-III and does not conform to the classification of these disorders in ICD-9-CM. When these departures occur, the DSM-III titles have been asterisked.

When a DSM-II category is listed without a corresponding DSM-III category, it indicates that the DSM-II category was not included in DSM-III. Likewise, when a DSM-III category is listed with no corresponding DSM-II category, it indicates that the DSM-III diagnostic concept was not included in DSM-II.

Included in the table are comments that attempt to explain the reasons for major changes in the DSM-II classification, terminology, or definitions of the categories. References are cited when the reason for a change is based on evidence cited in the literature or when the article referred to provides a fuller discussion of the rationale for the change.

The DSM-II classification follows, in order to assist the reader in locating specific categories, since the diagnostic categories are listed in the order in which they appear in DSM-II.

* Prepared by Robert L. Spitzer, M.D., Steven E. Hyler, M.D., and Janet B.W. Williams, M.S.W.

DSM-II

I Mental Retardation

Since the large majority of persons with borderline intellectual functioning (IQ=71-84) do not have significant impairment in adaptive behavior, this range of intellectual functioning is no longer included within Mental Retardation (1). DSM-III includes Borderline Intellectual Functioning as a V code for Conditions Not Attributable to a Mental

DSM-III

Mental Retardation

DSM-II

Disorder because of the frequent need to attend to this condition when planning treatment (2).

310.x	Borderline mental retardation	V62.89	Borderline intellectual functioning (included as a Condition not attributable to a mental disorder)
311.x	Mild mental retardation	317.00	Mild mental retardation
312.x	Moderate mental retardation	318.00	Moderate mental retardation
313.x	Severe mental retardation	318.10	Severe mental retardation
314.x	Profound mental retardation	318.20	Profound mental retardation
315.x	Unspecified mental retardation	319.00	Unspecified mental retardation

In DSM-II the fourth digit was used to note associated physical conditions. In DSM-III, the multiaxial system permits a more specific designation of associated physical conditions, noted on Axis III.

II Organic Brain Syndromes

The introduction to the Organic Brain Syndromes section of DSM-II implied the concept of a *single* organic brain syndrome with a limited number of manifestations. They were divided into the Psychoses and the Non-psychotic Organic Brain Syndromes. Because psychosis was defined in terms of "severity of functional impairment" and the "capacity to meet the ordinary demands of life," the psychotic-nonpsychotic distinction was difficult to make. Within the psychotic Organic Brain Syndromes, DSM-II retained the DSM-I distinction of acute vs. chronic brain syndrome. This distinction was based on the potential reversibility of the syndrome and not on course of the illness, as the terms "acute" and "chronic" are ordinarily used. This approach had many limitations; for example, it discouraged recognition of the possible reversibility of seemingly "chronic" brain syndromes, such as "reversible dementia" (3, 4, 5).

The DSM-III approach recognizes nine different organic brain syndromes: intoxication, withdrawal, delirium, dementia, amnestic syndrome, delusional syndrome, hallucinosis, affective syndrome, and personality syndrome. When the etiological factor is either associated with aging or is substance-induced, the etiological factor together with the specific organic brain syndrome constitutes the DSM-III Organic Mental Disorder. When there is some other etiology (e.g., pneumonia) or the etiology or pathophysiological process is unknown, the organic brain syndrome from Section 2 of the Organic Mental Disorders section of DSM-III is noted on Axis I and the specific physical disorder, if known, is noted on Axis III.

In DSM-II the category of "Drug Intoxication (other than alcohol)" did not allow for identifying the class of drug, or the more specific brain syndrome.

The DSM-II categories that have equivalent specific DSM-III categories are presented below.

The large number of specific DSM-III Organic Mental Disorders that have no direct DSM-II equivalents are not listed in this table.

290	Senile and presenile dementia	290.xx	Dementias arising in the senium and presenium
290.0	Senile dementia	290.xx	Primary degenerative dementia, senile onset
290.1	Presenile dementia	290.1x	Primary degenerative dementia, presenile onset

Fourth and fifth digits in DSM-III are used to code complications of delirium, delusional features or depressive features.

DSM-II**DSM-III****291 Alcoholic psychosis****291.0 Delirium tremens291.00 Alcohol withdrawal delirium****291.1 Korsakov's psychosis, alcoholic291.10 Alcohol amnestic disorder****291.2 Other alcoholic hallucinosis291.30 Alcohol hallucinosis****291.3 Alcohol paranoid state303.9x Alcohol Dependence* and a Paranoid Disorder***

Because there is no compelling evidence that a paranoid state due to chronic alcohol use is a distinct entity, DSM-III does not include a category for alcohol paranoid state.

291.4 Acute alcohol intoxication303.00 Alcohol intoxication***291.5 Alcoholic deterioration291.2x Dementia associated with alcoholism**

There is no compelling evidence that alcohol itself is the causative factor in Dementia in individuals with chronic Alcohol Dependence. For this reason, the ICD-9 term "Alcohol Dementia" is avoided since it implies that alcohol is known to be the causative factor (6).

291.6 Pathological intoxication291.40 Alcohol idiosyncratic intoxication

The DSM-III term is more precise than the DSM-II term.

293.0 Psychosis with cerebral arteriosclerosis290.4x Multi-infarct dementia

There is evidence that the dementia is related to the presence of multiple infarcts rather than the degree of cerebral arteriosclerosis (7, 8).

294.4 Psychosis with childbirth298.90 Atypical psychosis*

As with DSM-II, the DSM-III category is to be used only when no other psychotic disorder can be diagnosed. There is no compelling evidence that post-partum psychosis is a distinct entity.

II B. Non-psychotic Organic Brain Syndromes**309.13 Non-psychotic organic brain syndrome with alcohol (simple drunkenness)303.00 Alcohol intoxication*****309.30 Non-psychotic organic brain syndrome with circulatory disturbance290.4x Multi-infarct dementia***

309.60 Non-psychotic organic brain syndrome with senile or presenile brain disease	290.xx Primary degenerative dementia, senile onset
	290.1x Primary degenerative dementia, presenile onset

III B. Psychoses Not Attributed to Physical Conditions Listed Previously

DSM-III does not use "psychotic" as a fundamental basis for classifying the non-organic mental disorders in order to avoid classifying the Major Affective Disorders as psychotic, since such disorders usually do not have psychotic features.

SchizophreniaSchizophrenic Disorders

The DSM-III concept is more restrictive in order to identify a group that is more homogeneous in regard to differential response to somatic therapy, presence of a familial pattern, a tendency toward onset in early adult life, recurrence, and severe functional impairment (9, 10).

DSM-II

In DSM-II, two of the subtypes were defined by course, and the remaining subtypes were defined only by symptomatology. In DSM-III, the fourth digit is used to characterize symptomatology of the current episode, and the fifth digit is used to code the course of the illness as subchronic, chronic, subchronic with acute exacerbation, chronic with acute exacerbation, or in remission.

295.0 Schizophrenia, simple type 301.22 Schizotypal personality disorder*

The DSM-III concept of Schizophrenia requires the presence of psychotic features at some time during the illness. Furthermore, the validity of the category of Simple Schizophrenia has been questioned (11, 12). The closest approximation is Schizotypal Personality Disorder.

295.1 Schizophrenia, hebephrenic type 295.1x Schizophrenia, disorganized type

The term "hebephrenic" in this country has included only cases with regressive and silly behavior whereas the more common meaning has emphasized the disorganized aspect of the behavior (13).

295.2 Schizophrenia, catatonic type	}	 {	295.2x Schizophrenia, catatonic type
295.23 Schizophrenia, catatonic type, excited			296.4x Bipolar disorder, manic*
295.24 Schizophrenia, catatonic type, withdrawn			

Because of changes in the concepts of Schizophrenia and Affective Disorders, some cases of the DSM-II category of catatonic type will be diagnosed as having an Affective Disorder in DSM-III (14).

295.3 Schizophrenia, paranoid type 295.3x Schizophrenia, paranoid type

295.4 Acute schizophrenic episode	295.40 Schizophreniform disorder
	298.80 Brief reactive psychosis*
	295.70 Schizoaffective disorder*

The DSM-III category of Schizophrenia requires a duration of six months (including prodromal and residual phases), as this criterion defines a group that is more homogeneous with regard to familial pattern and course (15, 16, 17).

**295.5 Schizophrenia, latent type { 301.22 Schizotypal personality disorder
301.83 Borderline personality disorder**

The criteria for Schizotypal Personality Disorder were developed to identify a group of individuals clinically diagnosed as having Borderline Schizophrenia, a term included with the DSM-II category (18). However, this category is not included with the DSM-III category of Schizophrenia, since the category requires psychotic features at some time during the illness. Some individuals diagnosed as having Schizophrenia, Latent Type, in DSM-II may meet the criteria for Borderline Personality Disorder in DSM-III, instead of, or in addition to, Schizotypal Personality Disorder (18).

295.6 Schizophrenia, residual type	}	295.6x Schizophrenia, residual type
295.7 Schizophrenia, schizo-affective type		296.x4 Major affective disorder (depressed or manic) with psychotic features*
295.73 Schizophrenia, schizo-affective type, excited		296.40 Schizophreniform disorder
295.74 Schizophrenia, schizo-affective type, depressed		298.80 Brief reactive psychosis*
		295.xx Schizophrenia with superimposed Atypical affective disorder*
		295.70 Schizoaffective disorder

DSM-II

DSM-III

DSM-III acknowledges that certain bizarre psychotic symptoms are not incompatible with otherwise fully validated Affective Disorder (19). The other DSM-III categories are included here because of the heterogeneous nature of individuals previously classified as having Schizo-affective Schizophrenia (20, 21). In DSM-III Schizoaffective Disorder is a residual category, not included in Schizophrenia, and is to be used when there is uncertainty about the differential diagnosis between an Affective Disorder and either Schizophrenia, Schizophreniform Disorder, or Paranoid Disorder.

295.8 Schizophrenia, childhood type	{	299.0x Infantile autism 299.9x Childhood onset pervasive developmental disorder
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When children or adolescents have an illness that meets the criteria for Schizophrenia in DSM-III that diagnosis is given. There is evidence that the syndrome of Infantile Autism has little relationship to the psychotic disorders of adult life, particularly adult onset Schizophrenia (22). The relationship between the DSM-III category of Childhood Onset Pervasive Developmental Disorder and the psychotic disorders of adult life is unclear (23, 24, 25). The criteria for this disorder describe children who have been described by some clinicians as Childhood Schizophrenia, Childhood Psychosis, Atypical Children, and Symbiotic Psychosis. It is likely that some children with this disorder will indeed develop Schizophrenia as adults. However, there is currently no way of predicting which children will develop Schizophrenia as adults.

295.9 Schizophrenia, chronic undifferentiated type	295.9x Schizophrenia, undifferentiated type
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Major Affective Disorders	Major Affective Disorders
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In DSM-II Major Affective Disorders were included within the Non-organic Psychoses, and Affective Disorders that seemed to be "related directly to a precipitating life experience" were excluded. In contrast, the DSM-III category of Affective Disorders groups all the Affective Disorders together, regardless of the presence or absence of psychotic features or association with precipitating life experiences.

The DSM-III classification recognizes the heterogeneous nature of a major depressive episode (26) and uses the term melancholia to designate the subtype that tends to be more severe, associated with a constellation of characteristic symptoms, and is apparently particularly responsive to somatic therapy (27).

296.0 Involutional melancholia	296.23 Major depression, single episode with melancholia or 296.24 with psychotic features
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There is no compelling evidence that depression occurring in the involutional period is distinct from depression occurring at other stages of life (28).

296.1 Manic-depressive illness, manic type	296.4x Bipolar disorder, manic*
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Since virtually all individuals with manic episodes eventually develop depressive episodes, most investigators now conceptualize manic episodes as being subsumed under Bipolar Disorder (29). Therefore, in DSM-III, the diagnosis of Bipolar Disorder is made when there is a manic episode, whether or not there has been a depressive episode.

296.2 Manic-depressive illness, depressed type	Major depression 296.2x single episode 296.3x recurrent
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DSM-II**296.3 Manic-depressive illness, circular**

type	Bipolar disorder
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296.33 manic	296.4x manic
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296.34 depressed	296.5x depressed
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296.6x mixed

DSM-III

The DSM-II classification implied the unity of manic-depressive illness. DSM-III accepts the evidence pointing to the importance of the distinction between unipolar and bipolar forms of Affective Disorder (30, 31).

Paranoid States Paranoid Disorders

297.0 Paranoia	297.10 Paranoia
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297.1 Involuntional paranoid state	{ 297.10 Paranoia* 297.90 Atypical paranoid disorder*
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There is no compelling evidence that a Paranoid Disorder occurring in the involuntional period is distinct from Paranoid Disorders occurring in other periods of life (32).

297.30 Shared paranoid disorder

This category is the traditional category of Folie à deux. The justification for its inclusion in DSM-III, despite its rarity, rests on the distinct clinical picture and the treatment implications (33, 34).

298.30 Acute paranoid disorder

This category permits the identification of the most common form of Paranoid Disorder which is of acute onset and of brief duration (32).

298.0 Psychotic depressive reaction	296.24 Major depression, single episode, with psychotic features,* with coding of severity of psychosocial stressor on Axis IV
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There is no compelling evidence that once a Major Depression has developed, its course and response to treatment are affected by whether or not its onset was associated with a stressor.

IV Neuroses

In DSM-II disorders in which the "chief characteristic" was anxiety, whether "felt and expressed directly" or "controlled unconsciously and automatically by conversion, displacement and various other psychological mechanisms" were grouped together as Neuroses. In contrast, in DSM-III the disorders in which anxiety is experienced directly are grouped together in the class of Anxiety Disorders. The other DSM-II neuroses are distributed among other classes, each defined by shared symptoms or other descriptive characteristics. So that one can identify the categories that in DSM-II were grouped together in the class of Neuroses, the DSM-II terms are included separately in parentheses after the corresponding DSM-III categories. (See DSM-III classification.)

300.0 Anxiety neurosis	{ 300.01 Panic disorder 300.02 Generalized anxiety disorder
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There is compelling evidence that Panic Disorder, as a distinct entity, has differential treatment response as compared with other disorders in which anxiety is prominent (35, 36).

DSM-II

DSM-III

300.13 Hysterical neurosis

In DSM-III, the concept and the term "hysteria" have been avoided. Instead, the multiple meanings of the term have been included within new categories, such as Somatoform Disorders and Dissociative Disorders (37).

300.1 Hysterical neurosis, conversion type { **300.11 Conversion disorder**
307.80 Psychogenic pain disorder*

This latter DSM-III category permits the identification of individuals whose predominant complaint is pain, apparently of psychogenic origin (38).

300.14 Hysterical neurosis, dissociative type { **300.12 Psychogenic amnesia**
300.13 Psychogenic fugue
300.14 Multiple personality
307.46 Sleepwalking disorder*
(in the childhood section)

In DSM-III, the four disorders included in the DSM-II description are defined as separate disorders because of differing clinical pictures, predisposing factors, and course (39, 40). The first three disorders are included within the Dissociative Disorders. Sleepwalking Disorder is listed in the section Disorders Usually First Evident in Infancy, Childhood or Adolescence and is defined as a disturbance of a particular stage of sleep (41, 42).

Phobic disorders

300.2 Phobic neurosis { **300.21 Agoraphobia with panic attacks**
300.22 Agoraphobia without panic attacks
300.23 Social phobia
300.29 Simple phobia
309.21 Separation anxiety disorder*
(in the childhood section)

DSM-III subdivides phobias into separate categories because of differing clinical pictures, ages at onset, and differential treatment responses (43). Even though Separation Anxiety Disorder is a form of Phobia, because it characteristically begins in infancy or childhood, and rarely persists into adulthood, it is classified in the section "Disorders Usually First Evident in Infancy, Childhood or Adolescence" (44, 45).

300.3 Obsessive compulsive neurosis **300.30 Obsessive compulsive disorder**

300.4 Depressive neurosis { **Major depression**
296.22 single episode, without melancholia
296.32 recurrent, without melancholia
300.40 Dysthymic disorder
309.00 Adjustment disorder with depressed mood

The DSM-II category was defined merely as "an excessive reaction of depression due to an internal conflict or to an identifiable event . . ." For this reason, it was applied to a heterogeneous group of conditions (46). The three major conditions to which it was applied have been defined descriptively without reference to etiology. When an "identifiable event" is judged to have contributed to the development of the illness, this factor can be noted on Axis IV.

300.5 Neurasthenic neurosis

This DSM-II category was rarely used.

DSM-II	DSM-III
300.6 Depersonalization neurosis	300.60 Depersonalization disorder

The DSM-III category is included within the class of Dissociative Disorders, even though this is controversial, because the feeling of one's own reality, a component of identity, is lost (47).

300.7 Hypochondriacal neurosis	300.70 Hypochondriasis
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Hypochondriasis is included within the class of Somatoform Disorders because of the presentation of symptoms suggestive of physical disorder.

300.81 Somatization disorder

This disorder has been described in the literature as either "Hysteria" or "Briquet's Syndrome" and validity data have been gathered in a series of studies (48).

Post-traumatic stress disorder

- 308.30 acute**
- 309.81 chronic or delayed**

This DSM-III category used to be referred to as Traumatic Neurosis (49). Its subdivision into acute and chronic forms is justified by longitudinal studies showing differential outcomes for the two forms (50, 51, 52).

V Personality Disorders	Personality Disorders
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Personality Disorders in DSM-III are coded on Axis II in order to insure that they are not overlooked when attention is directed to the usually more florid Axis I disorder.

DSM-III recognizes the distinction between personality traits and personality disorders. For this reason, the term "disorder" appears in the diagnostic term. Although prominent personality traits may be noted on Axis II, they are not given code numbers since they do not represent mental disorders.

301.0 Paranoid personality	301.00 Paranoid personality disorder
301.1 Cyclothymic personality	301.13 Cyclothymic disorder

Cyclothymic disorder is included in DSM-III within the Affective Disorders rather than the Personality Disorders, because of evidence that it is related to Bipolar Disorder (53).

301.2 Schizoid personality	{ 301.22 Schizotypal personality disorder
	{ 301.20 Schizoid personality disorder
	{ 301.82 Avoidant personality disorder*

The DSM-II category included "shyness, over-sensitivity, seclusiveness, avoidance of close or competitive relationships, and often eccentricity." In DSM-III Schizotypal Personality Disorder describes individuals with the eccentric features referred to in the DSM-II description.

The criteria for Schizotypal Personality Disorder were developed to identify individuals who had been described as having Borderline Schizophrenia (18). There is evidence that Chronic Schizophrenia is more common among family members of individuals who were described as having Borderline Schizophrenia than in the general population (54). The distinction between the DSM-III categories of Schizoid and Avoidant Personality Disorders is based on whether or not there is a defect in the motivation and capacity for emotional involvement (55). It is expected that this descriptive distinction will have therapeutic and prognostic implications.

DSM-II

DSM-III

301.3 Explosive personality 312.34 Intermittent explosive disorder*

Because the explosive behavior is, by definition, in contrast to the individual's usual behavior, the disorder in DSM-III is not classified among the personality disorders.

301.4 Obsessive compulsive personality 301.40 Compulsive personality disorder

The DSM-III label omits the term "obsessive" in order to avoid confusion with Obsessive Compulsive Disorder.

301.5 Hysterical personality 301.50 Histrionic personality disorder

The term "hysterical" has many irrelevant historical connotations and suggests a relationship to conversion symptoms (56, 57). The essential feature of this disorder, as described in both DSM-II and DSM-III, is the histrionic pattern of behavior.

301.6 Asthenic personality

This DSM-II category was rarely used.

301.7 Antisocial personality 301.70 Antisocial personality disorder

The DSM-III description and criteria are based on longitudinal studies of children whose antisocial behavior persisted into adult life (58).

301.81 Passive-aggressive personality 301.84 Passive-aggressive personality disorder

301.82 Inadequate personality

This DSM-II category was defined primarily in terms of functional impairment, rather than a distinctive behavior pattern.

301.81 Narcissistic personality disorder

In recent years considerable attention has been given to narcissistic disturbances in personality in the psychoanalytic literature (59, 60).

301.83 Borderline personality disorder

This category identifies a constellation of relatively enduring personality features of instability and vulnerability believed to have important treatment and outcome implications (61). The criteria for this category are supported by the results of a factor analytic study of symptom data of patients clinically designated as having a borderline condition and are consistent with the literature describing borderline conditions (18). Some of these individuals were diagnosed as having Schizophrenia, latent type, in DSM-II.

301.60 Dependent personality disorder

This category is roughly equivalent to the DSM-I category of Passive aggressive personality, dependent type.

Sexual Deviations Paraphilias

The term "paraphilias" is preferable to "sexual deviations" in that it correctly emphasizes that the deviation (para) is in that to which the individual is attracted (philia).

DSM-II

**302.0 Homosexuality (replaced in 1973
with Sexual orientation dis-
turbance)**

DSM-III

**302.00 Ego-dystonic homosexuality (in-
cluded in Other Psychosexual
Disorders)**

Whether or not homosexuality per se should be classified as a mental disorder has been the focus of considerable controversy (62). In December 1973, the Board of Trustees of the American Psychiatric Association voted to eliminate homosexuality per se as a mental disorder and to substitute a new category, Sexual Orientation Disturbance, reserved for those homosexuals who are "disturbed by, in conflict with, or wish to change their sexual orientation." This change appeared in the seventh and subsequent printings of DSM-II.

The removal of homosexuality per se from DSM-II was supported by the following rationale: The crucial issue in determining whether or not homosexuality per se should be regarded as a mental disorder is not the etiology of the condition, but its consequences and the definition of mental disorder (63). A significant proportion of homosexuals are apparently satisfied with their sexual orientation, show no significant signs of manifest psychopathology (unless homosexuality, by itself, is considered psychopathology), and are able to function socially and occupationally with no impairment (64, 65, 66, 67). If one uses the criteria of *distress* or *disability*, homosexuality per se is not a mental disorder. If one uses the criterion of *inherent disadvantage*, it is not at all clear that homosexuality is a disadvantage in all cultures or subcultures (68, 69).

In DSM-III, the category of Ego-dystonic Homosexuality is a modification of the DSM-II category of Sexual Orientation Disturbance. The change in terminology was made to make it clear that the category is limited to individuals with a homosexual arousal pattern. Changes in the definition of the category emphasize the impairment in heterosexual functioning. Ego-dystonic Homosexuality is not included as a Paraphilia in DSM-III, in contrast to the inclusion of both Homosexuality and Sexual Orientation Disturbance in DSM-II as Sexual Deviations, because in DSM-III the Paraphilias are limited to conditions that are associated with (1) preference for the use of a non-human object for sexual arousal, (2) repetitive sexual activity with humans involving real or simulated suffering or humiliation, or (3) repetitive sexual activity with non-consenting or inappropriate partners. In contrast, the DSM-II category of Sexual Deviations also included those "Individuals whose sexual interests are directed primarily toward objects other than people of the opposite sex."

302.1 Fetishism	302.81 Fetishism
302.2 Pedophilia	302.20 Pedophilia
302.3 Transvestism	302.30 Transvestism
302.4 Exhibitionism	302.40 Exhibitionism
302.5 Voyeurism	302.82 Voyeurism
302.6 Sadism	302.84 Sexual sadism
302.7 Masochism	302.83 Sexual masochism

The DSM-III terms for the last two categories above avoid any confusion with non-sexual meanings of these terms.

302.10 Zoophilia (70)

Gender Identity Disorders

- 302.5x Transsexualism (71, 72)**
- 302.60 Gender identity disorder of
childhood (73)**

DSM-II**DSM-III****Psychosexual Dysfunctions**

- 302.71** Inhibited sexual desire
- 302.72** Inhibited sexual excitement
- 302.73** Inhibited female orgasm
- 302.74** Inhibited male orgasm
- 302.75** Premature ejaculation
- 302.76** Functional dyspareunia
- 306.51** Functional vaginismus

DSM-II listed, as examples of Psychophysiological Genito-urinary Disorder, both Dyspareunia and Impotence, whose DSM-III equivalents are Functional Dyspareunia and Inhibited Sexual Excitement (in a male). The justification for including the other specific Psychosexual Dysfunctions rests on their clinical importance and differential treatments (74, 75).

Alcoholism **Alcohol Abuse and Dependence (included within Substance Use Disorders)**

In DSM-III, the equivalent categories are included within the Substance Use Disorders to emphasize the fact that the effects of the maladaptive use of alcohol are similar to the effects of the maladaptive use of other substances of potential abuse and dependence.

- | | | | | |
|--------------|------------------------------------|-------|---------------|-----------------------------------|
| 303.0 | Episodic excessive drinking | | 305.02 | Alcohol abuse, episodic* |
| 303.1 | Habitual excessive drinking | | 305.01 | Alcohol abuse, continuous* |
| 303.2 | Alcohol addiction | | 303.9x | Alcohol dependence |

In DSM-III, for each Substance Use Disorder, the course of the illness may be noted in the fifth digit as continuous, episodic, or in remission.

Drug Dependence **Substance Use Disorders**

The DSM-II Drug Dependence category included what in DSM-III is referred to as Substance Abuse and Substance Dependence. In DSM-II, the term "dependence" included both psychological dependence and physiological dependence. In DSM-III, dependence is used only in the physiological sense and requires evidence of either tolerance or withdrawal. The DSM-II Drug Dependence category specifically included alcohol (coded separately) and tobacco, whereas these substances are both included within the DSM-III Substance Use Disorders.

- | | | | | |
|--------------|--|-------|---------------|--|
| 304.0 | Drug dependence, opium, opium alkaloids and their derivatives | } | 305.5x | Opioid abuse |
| 304.1 | Drug dependence, synthetic analgesics with morphine-like effects | | 304.0x | Opioid dependence |
| 304.2 | Drug dependence, barbiturates | } | 305.4x | Barbiturate or similarly acting sedative or hypnotic abuse |
| 304.3 | Drug dependence, other hypnotics and sedatives or "tranquilizers" | | 304.1x | Barbiturate or similarly acting sedative or hypnotic dependence |
| 304.4 | Drug dependence, cocaine | | 305.6x | Cocaine abuse |
| 304.5 | Drug dependence, Cannabis sativa (hashish, marihuana) | | 305.2x | Cannabis abuse |
| | | | 304.3x | Cannabis dependence |

The existence and significance of tolerance or withdrawal with regular heavy use of cannabis (Cannabis Dependence) is controversial (76, 77).

DSM-II

**304.6 Drug dependence, other psycho-
stimulants (amphetamines, etc.) . .**

DSM-III

**305.7x Amphetamine or similarly act-
ing sympathomimetic abuse**
**304.4x Amphetamine or similarly act-
ing sympathomimetic
dependence**

304.7 Drug dependence, hallucinogens . . . 305.3x Hallucinogen abuse

No withdrawal syndrome from hallucinogens has ever been described.

**305.9x Phencyclidine (PCP) or similarly
acting arylcyclohexylamine
abuse**

This relatively new substance of abuse is distinguished from hallucinogens, despite some similarities in their effects (78).

305.1x Tobacco dependence

The justification for the inclusion of Tobacco Dependence in DSM-III (as it is in the Ninth Revision of the International Classification of Diseases) rests on the serious medical complications of long-term use (79, 80, 81). It could be argued that the absence of both an intoxication state and the kinds of social complications associated with other substances of dependence speak for classifying Tobacco Dependence as a physical disorder, not a mental disorder. However, the behavioral manifestations of the dependence (inability to control use) and the withdrawal syndrome are by no means inconsequential. Furthermore, by tradition, substance dependence is classified as a mental disorder.

**304.7x Dependence on combination of
opioid and other non-
alcoholic substance**

**304.8x Dependence on combination of
substances, excluding opioids
and alcohol**

These two categories are necessary to indicate poly-substance use when it is not possible to identify all the specific substances involved.

**VI Psychophysiologic Disorders 316.00 Psychological Factors Affecting
Physical Condition***

The DSM-II approach to the classification of so-called "psychophysiologic" or "psychosomatic disorders" had several practical and theoretical shortcomings. The categories of psychophysiologic disorders were rarely used. The choice between a psychophysiological diagnosis and an "organic" diagnosis tended to be made idiosyncratically. The DSM-II approach did not encourage collaboration between psychiatrists and other medical specialists. The theoretical basis for the category perpetuated a simplistic, uncausal concept about disease etiology. The DSM-III approach attempts to overcome these shortcomings by the use of the multiaxial system. When the clinician judges that a psychological factor is associated with either the initiation or exacerbation of a physical condition or disorder, the category of Psychological Factors Affecting Physical Condition is noted on Axis I. The physical condition or disorder is noted on Axis III. The limitations of the DSM-II approach and the potential advantages of this approach are discussed more fully elsewhere (82, 83).

VII Special Symptoms

This section of DSM-II was intended for "discrete, specific symptoms" as distinguished

DSM-II

DSM-III

from mental disorders. The names of the symptoms were listed with no descriptions. In DSM-III most of these "symptoms" are included as mental disorders because of their syndromal nature (e.g., Anorexia Nervosa), or because they represent a distinct clinical pattern (e.g., Functional Enuresis).

306.0 Speech disturbance 307.00 Stuttering

The only speech disturbance included in DSM-III is Stuttering. Other speech disturbances are unlikely to come to the attention of a mental health professional.

Specific Developmental Disorders

306.1 Specific learning disturbance	315.00	Developmental reading disorder
	315.10	Developmental arithmetic disorder
	315.31	Developmental language disorder
	315.39	Developmental articulation disorder
	315.50	Mixed specific developmental disorder

The DSM-III term, Specific Developmental Disorders, indicates that these disorders are characterized by specific delays in development. Because of differential treatment implications they are divided according to the predominant area of functioning that is impaired (84, 85, 86, 87, 88). They are coded on Axis II in order to insure that they are considered when the individual has a more florid Axis I disorder.

Stereotyped movement disorders

306.2 Tic	307.21	Transient tic disorder
	307.22	Chronic motor tic disorder
	307.23	Tourette's disorder

The three major forms of Tic Disorders are described separately because of differing clinical pictures, courses, and treatment implications (89).

306.4 Disorders of sleep	307.46*	Sleepwalking disorder
	307.46*	Sleep terror disorder

Of the many disorders of sleep, DSM-III includes only these two because of their marked behavioral manifestations, because of the frequency with which they come to the attention of a mental health professional, and because, by tradition, they are thought of as mental disorders (41, 90, 91). (A new classification of Sleep and Arousal Disorders appears in Appendix E.)

Eating disorders

306.5 Feeding disturbance	307.10	Anorexia nervosa
	307.51	Bulimia
	307.52	Pica
	307.53	Rumination disorder of infancy

The Eating Disorders are described separately because of differing clinical pictures, courses, and treatment implications (92).

306.6 Enuresis	307.60	Functional enuresis
306.7 Encopresis	307.70	Functional encopresis

The DSM-III terms emphasize the exclusion of known physical etiology.

* Not an error. The same code is used to maintain compatibility with ICD-9-CM.

DSM-II

306.80 Cephalalgia

It is not clear what was included within this DSM-II category.

DSM-III

VIII Transient Situational Disturbances Adjustment Disorder

The DSM-II category was "reserved for more or less transient disorders of any severity (including those of psychotic proportions) that occur in individuals without any apparent underlying mental disorders . . ."

The DSM-III category of Adjustment Disorder excludes reactions of psychotic proportion since they are adequately classified elsewhere. Adjustment Disorder can be given as an additional diagnosis to an individual with an underlying disorder, e.g., a Personality Disorder, since there is evidence that individuals with Personality Disorders are particularly vulnerable to stress (93).

The DSM-II classification of Transient Situational Disturbances by developmental stage, infancy to late life, offered no information about the manifestations of the disturbance that would be of importance in planning treatment. For this reason, in DSM-III Adjustment Disorder is subtyped by predominant symptomatology.

307.0 Adjustment reaction of infancy	}	309.00 with depressed mood
307.1 Adjustment reaction of childhood		309.24 with anxious mood
307.2 Adjustment reaction of adolescence		309.28 with mixed emotional features
307.3 Adjustment reaction of adult life		309.30 with disturbance of conduct
307.4 Adjustment reaction of late life		309.40 with mixed disturbance of emotions and conduct
		309.23 with work (or academic) inhibition
		309.83 with withdrawal

IX Behavior Disorders of Childhood and Adolescence

Disorders Usually First Evident in Infancy, Childhood, or Adolescence

The DSM-II category was limited to a small number of categories appropriate only for children or adolescents. They were conceptualized as midway between Transient Situational Disturbances, on one hand, and Psychoses, Neuroses and Personality Disorders, on the other hand, in terms of stability and resistance to treatment. In contrast, the DSM-III section includes a large number of diagnoses of varying degrees of severity and stability. Furthermore, when appropriate, some of the diagnoses may be given to adults.

308.0 Hyperkinetic reaction of childhood (or adolescence)	314.01 Attention deficit disorder, with hyperactivity
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The DSM-III term is used to reflect the observation that attentional difficulties are prominent and virtually always present in hyperkinetic children. Alternative terms for this disorder, such as Minimal Brain Dysfunction, are based on unproven assumptions (94, 95).

308.1 Withdrawing reaction of childhood (or adolescence)	}	313.21 Avoidant disorder of childhood or adolescence
		313.22 Schizoid disorder of childhood or adolescence

DSM-II

DSM-III

The distinction between Avoidant and Schizoid Disorders of Childhood or Adolescence is based on whether or not there is a defect in the motivation and capacity for emotional involvement. It is expected that this descriptive distinction will have therapeutic and prognostic implications.

308.2 Overanxious reaction of childhood (or adolescence)	313.00 Overanxious disorder
308.3 Runaway reaction of childhood (or adolescence)	312.10 Conduct disorder, under-socialized, nonaggressive
308.4 Unsocialized aggressive reaction of childhood (or adolescence)	312.00 Conduct disorder, under-socialized, aggressive
308.5 Group delinquent reaction of childhood (or adolescence)	312.23 Conduct disorder, socialized, aggressive
	312.21 Conduct disorder, socialized, nonaggressive

In DSM-III the category of Conduct Disorder includes cases in which there is a repetitive and persistent pattern of aggressive or non-aggressive conduct in which either the rights of others or major age-appropriate societal norms or rules are violated. The subdivision of Conduct Disorder is controversial. There is evidence that the frequency and the variety of childhood antisocial behaviors are predictive of adult antisocial behavior (96, 97, 98). At the same time, there is evidence that the presence or the absence of adequate social attachments (socialization) in children with antisocial behavior has prognostic significance (99, 100). The DSM-III approach divides Conduct Disorder into four subtypes. The justification for the aggressive-nonaggressive distinction rests on an obvious difference in clinical picture, with management considerations. The socialized-undersocialized dichotomy is likely to have treatment implications, even if the prognostic implications are still unclear.

314.00 Attention deficit disorder, without hyperactivity

It is clinically recognized that some children with attentional difficulties have never had concomitant hyperactivity (101).

314.80 Attention deficit disorder, residual type

There is evidence that some individuals who, as children, had Attention Deficit Disorder with Hyperactivity, in adolescence or adulthood no longer have hyperactivity, although they continue to have attentional difficulties (102, 103).

313.89 Reactive attachment disorder of infancy

This category has been described in the literature under a variety of names, including "failure to thrive without organic basis" (104, 105).

313.23 Elective mutism

This is a well-recognized syndrome (106, 107).

313.81 Oppositional disorder

This category was included as a Personality Disorder in a classification of disorders in childhood proposed by the Group for the Advancement of Psychiatry (108). It is

DSM-II

included in DSM-III in modified form and is to be distinguished from Passive-Aggressive Personality Disorder and from Conduct Disorder, Socialized, Nonaggressive.

DSM-III

313.82 Identity disorder

There is a large literature on identity problems in adolescence (109, 110). This category is to be distinguished from the DSM-III category of Borderline Personality Disorder.

Disorders with no corresponding DSM-II categories

Factitious Disorders

300.16 Factitious disorder with psychological symptoms

301.51 Chronic factitious disorder with physical symptoms (Munchausen syndrome)

The prototype of Factitious Disorders, Munchausen Syndrome, has been recognized in the literature (111, 112), as have other factitious illnesses with physical symptoms, such as Factitious Dermatitis (113). Despite the rarity of these disorders, it seems useful to distinguish them as a class of disorders (37).

Disorders of Impulse Control Not Elsewhere Classified

312.31 Pathological gambling

312.32 Kleptomania

312.33 Pyromania

312.35 Isolated explosive disorder

These disorders have been recognized in the literature as having distinct clinical pictures with differing treatment implications and with obvious relevance to forensic issues (114, 115, 116, 117, 118). In DSM-III, this class also includes Intermittent Explosive Disorder, which is roughly equivalent to the DSM-II category of Explosive Personality Disorder, as noted above.

X Conditions Without Manifest Psychiatric Disorder and Non-Specific Conditions

316 Social Maladjustments Without Manifest Psychiatric Disorder

V Codes for Conditions Not Attributable to a Mental Disorder That Are a Focus of Attention or Treatment

The DSM-II category was limited to "individuals who are psychiatrically normal but who nevertheless have severe enough problems to warrant examination by a psychiatrist." No definition of normality was provided. As the DSM-III name implies, these categories may be given to an individual who has a mental disorder, as long as the condition itself is not attributable to a mental disorder.

316.0 Marital maladjustment **V61.10**

Marital problem

316.1 Social maladjustment **V62.89**

Phase of life problem or other life circumstance problem*

316.2 Occupational maladjustment **V62.20**

Occupational problem

316.3 Dyssocial behavior

V71.01

Adult antisocial behavior

V71.02

Childhood or adolescent antisocial behavior

V65.20

Malingering

Malingering (119) has been included because of its obvious relevance to forensic psychiatry.

DSM-II

DSM-III

- V62.30 Academic problem
- V62.82 Uncomplicated bereavement
- V15.81 Noncompliance with medical treatment
- V61.20 Parent-child problem
- V61.80 Other specified family circumstance
- V62.81 Other interpersonal problem

It seems useful to be able to distinguish these problems from mental disorders. The study of bereavement has made it possible to distinguish it from Major Depression (120, 121).

317 Non-specific conditions	{	300.90	Unspecified mental disorder (non-psychotic)
		298.90	Atypical psychosis
318 No mental disorder	{	V71.09	No diagnosis or condition on Axis I
		V71.09	No diagnosis on Axis II
319 Diagnosis deferred	{	799.90	Diagnosis or condition deferred on Axis I
		799.90	Diagnosis deferred on Axis II

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DSM-II DIAGNOSES and CODES**MENTAL RETARDATION**

- 310. Borderline
- 311. Mild
- 312. Moderate
- 313. Severe
- 314. Profound
- 315. Unspecified

With each: Following or associated with

- .00 Infection or intoxication
- .10 Trauma or physical agent
- .20 Disorders of metabolism, growth or nutrition
- .30 Gross brain disease (postnatal)
- .40 Unknown prenatal influence
- .50 Chromosomal abnormality
- .60 Prematurity
- .70 Major psychiatric disorder
- .80 Psycho-social (environmental) deprivation
- .90 Other condition

ORGANIC BRAIN SYNDROMES (OBS)**A PSYCHOSES****Senile and pre-senile dementia**

- 290.00 Senile dementia
- 290.10 Pre-senile dementia

Alcoholic psychosis

- 291.00 Delirium tremens
- 291.10 Korsakov's psychosis
- 291.20 Other alcoholic hallucinosis
- 291.30 Alcohol paranoid state
- 291.40 Acute alcohol intoxication
- 291.50 Alcoholic deterioration
- 291.60 Pathological intoxication
- 291.90 Other alcoholic psychosis

Psychosis associated with intracranial infection

- 292.00 General paralysis
- 292.10 Syphilis of central nervous system
- 292.20 Epidemic encephalitis
- 292.30 Other and unspecified encephalitis
- 292.90 Other intracranial infection

Psychosis associated with other cerebral condition

- 293.00 Cerebral arteriosclerosis
- 293.10 Other cerebrovascular disturbance
- 293.20 Epilepsy

- 293.30 Intracranial neoplasm
- 293.40 Degenerative disease of the CNS
- 293.50 Brain trauma
- 293.90 Other cerebral condition

Psychosis associated with other physical condition

- 294.00 Endocrine disorder
- 294.10 Metabolic or nutritional disorder
- 294.20 Systemic infection
- 294.30 Drug or poison intoxication (other than alcohol)
- 294.40 Childbirth
- 294.80 Other and unspecified physical condition

B NON-PSYCHOTIC OBS

- 309.00 Intracranial infection
- 309.13 Alcohol (simple drunkenness)
- 309.14 Other drug, poison, or systemic intoxication
- 309.20 Brain trauma
- 309.30 Circulatory disturbance
- 309.40 Epilepsy
- 309.50 Disturbance of metabolism, growth or nutrition
- 309.60 Senile or pre-senile brain disease
- 309.70 Intracranial neoplasm
- 309.80 Degenerative disease of the CNS
- 309.90 Other physical condition

PSYCHOSES NOT ATTRIBUTED TO PHYSICAL CONDITIONS LISTED PREVIOUSLY**Schizophrenia**

- 295.00 Simple
- 295.10 Hebephrenic
- 295.20 Catatonic
- 295.23 Catatonic type, excited
- 295.24 Catatonic type, withdrawn
- 295.30 Paranoid
- 295.40 Acute schizophrenic episode
- 295.50 Latent
- 295.60 Residual
- 295.70 Schizo-affective
- 295.73 Schizo-affective, excited
- 295.74 Schizo-affective, depressed
- 295.80 Childhood
- 295.90 Chronic undifferentiated
- 295.99 Other schizophrenic

Major affective disorders

- 296.00 Involutional melancholia
- 296.10 Manic-depressive illness, manic
- 296.20 Manic-depressive illness, depressed
- 296.30 Manic-depressive illness, circular
- 296.33 Manic-depressive, circular, manic
- 296.34 Manic-depressive, circular, depressed
- 296.80 Other major affective disorder

Paranoid states

- 297.00 Paranoia
- 297.10 Involutional paranoid state
- 297.90 Other paranoid state

Other psychoses

- 298.00 Psychotic depressive reaction

NEUROSES

- 300.00 Anxiety
- 300.10 Hysterical
- 300.13 Hysterical, conversion type
- 300.14 Hysterical, dissociative type
- 300.20 Phobic
- 300.30 Obsessive compulsive
- 300.40 Depressive
- 300.50 Neurasthenic
- 300.60 Depersonalization
- 300.70 Hypochondriacal
- 300.80 Other neurosis

PERSONALITY DISORDERS AND CERTAIN OTHER NON-PSYCHOTIC MENTAL DISORDERS**Personality disorders**

- 301.00 Paranoid
- 301.10 Cyclothymic
- 301.20 Schizoid
- 301.30 Explosive
- 301.40 Obsessive compulsive
- 301.50 Hysterical
- 301.60 Asthenic
- 301.70 Antisocial
- 301.81 Passive-aggressive
- 301.82 Inadequate
- 301.89 Other specified types

Sexual deviation

- 302.00 Homosexuality
- 302.10 Fetishism

- 302.20 Pedophilia
- 302.30 Transvestism
- 302.40 Exhibitionism
- 302.50 Voyeurism
- 302.60 Sadism
- 302.70 Masochism
- 302.80 Other sexual deviation

Alcoholism

- 303.00 Episodic excessive drinking
- 303.10 Habitual excessive drinking
- 303.20 Alcohol addiction
- 303.90 Other alcoholism

Drug dependence

- 304.00 Opium, opium alkaloids and their derivatives
- 304.10 Synthetic analgesics with morphine-like effects
- 304.20 Barbiturates
- 304.30 Other hypnotics and sedatives or "tranquilizers"
- 304.40 Cocaine
- 304.50 Cannabis sativa (hashish, marihuana)
- 304.60 Other psycho-stimulants
- 304.70 Hallucinogens
- 304.80 Other drug dependence

PSYCHOPHYSIOLOGIC DISORDERS

- 305.00 Skin
- 305.10 Musculoskeletal
- 305.20 Respiratory
- 305.30 Cardiovascular
- 305.40 Hemic and lymphatic
- 305.50 Gastro-intestinal
- 305.60 Genito-urinary
- 305.70 Endocrine
- 305.80 Organ of special sense
- 305.90 Other type

SPECIAL SYMPTOMS

- 306.00 Speech disturbance
- 306.10 Specific learning disturbance
- 306.20 Tic
- 306.30 Other psychomotor disorder
- 306.40 Disorders of sleep
- 306.50 Feeding disturbance
- 306.60 Enuresis
- 306.70 Encopresis
- 306.80 Cephalalgia
- 306.90 Other special symptom

**TRANSIENT SITUATIONAL
DISTURBANCES**

- 307.00 Adjustment reaction of infancy
- 307.10 Adjustment reaction of childhood
- 307.20 Adjustment reaction of adolescence
- 307.30 Adjustment reaction of adult life
- 307.40 Adjustment reaction of late life

**BEHAVIOR DISORDERS OF CHILD-
HOOD AND ADOLESCENCE**

- 308.00 Hyperkinetic reaction
- 308.10 Withdrawing reaction
- 308.20 Overanxious reaction
- 308.30 Runaway reaction
- 308.40 Unsocialized aggressive reaction
- 308.50 Group delinquent reaction
- 308.90 Other reaction

**CONDITIONS WITHOUT MANIFEST
PSYCHIATRIC DISORDER AND
NON-SPECIFIC CONDITIONS**

**Social maladjustment without manifest
psychiatric disorder**

- 316.00 Marital maladjustment
- 316.10 Social maladjustment
- 316.20 Occupational maladjustment
- 316.30 Dyssocial behavior
- 316.90 Other social maladjustment

Non-specific conditions

- 317.00 Non-specific conditions

No mental disorder

- 318.00 No mental disorder

**NON-DIAGNOSTIC TERMS FOR
ADMINISTRATIVE USE**

- 319.00 Diagnosis deferred
- 319.10 Boarder
- 319.20 Experiment only
- 319.90 Other

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**Historical Review, and
Mental Disorders Sections
of ICD-9 and ICD-9-CM**

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Appendix D: Historical Review, and Mental Disorders Sections of ICD-9 and ICD-9-CM

HISTORICAL REVIEW*

This historical review covers the activities of the World Health Organization (WHO) in developing the classification of mental disorders (Section V) of the Ninth Revision of the International Classification of Diseases (ICD-9), and some of the major changes from ICD-8. In addition, the clinical modification of ICD-9 (ICD-9-CM) that was introduced in the United States, and the relationship between it and the DSM-III classification are discussed.

Purposes and Uses of the ICD

The ICD is a statistical classification not only of mental disorders but of diseases and other morbid conditions; complications of pregnancy, childbirth, and the puerperium; congenital abnormalities; causes of perinatal morbidity and mortality; accidents, poisonings, and violence; and symptoms, signs, and ill-defined conditions. Its principal use is in the classification of morbidity and mortality information for statistical purposes, as the unabridged title of the classification makes quite clear: *The International Statistical Classification of Diseases, Injuries, and Causes of Death*.

The ICD has also been adapted for use as a nomenclature of diseases for indexing medical records. The basic purpose of such indexing is to facilitate retrieval of medical records for a variety of purposes (for example, studies of management of patients with specific conditions; follow-up studies of patients with specific diseases who have undergone various operative and therapeutic procedures). Its limitations for these purposes led to the development of the ICD-9-CM in the United States—a subject to be discussed below.

It is essential to keep in mind that the ICD is a statistical classification of diseases, not a nomenclature of diseases. The distinction between these two is important. A nomenclature of diseases is a list or catalogue of approved terms for describing and recording clinical and pathological observations. To serve its full function it must be sufficiently extensive so that any pathological condition can be accurately recorded. As medical science advances, a nomenclature must expand to include the new terms necessary to record new observations. In contrast, a statistical classification indicates the relationship between diagnostic

* Prepared by Morton Kramer, Sc.D.

categories and must be confined to a limited number of categories that encompass the entire range of diseases and morbid conditions.

Organizational Arrangement of ICD-9

The ICD is organized into 17 major sections, each of which is devoted to a specific set of conditions (Table 1). Each of these major sections is subdivided into a defined set of categories, each identified by three digits ranging from 001 to 999. To provide greater detail, each such category is further divided into additional subcategories by a fourth digit (.0 to .9). Table 1 shows the number of three-digit categories allotted to each major section. Only 30 are allotted to mental disorders, so that all mental disorders must be classified within these 30 categories and their fourth-digit subdivisions.

The structure of the classification is such that the axes of classification are not consistent within each of the 17 major sections. In some of these categories (e.g., diseases of the respiratory system) the primary axis is topographical; less frequently it is etiological (e.g., infectious diseases) or situational (e.g., complications of pregnancy). In other sections, still other primary axes are used, reflecting the fact that the ICD provides a pragmatic classification that can be used for a variety of purposes.

There are also two supplementary chapters: one for classification of external causes of injury and poisoning (the E codes), and the other for classification of factors influencing health status and contact with health services (the V codes). Both of these classifications contain items of relevance to agencies and facilities that provide mental health services.

Revisions of the ICD

The ICD is revised regularly, at approximately ten-year intervals. This pattern was initiated with the First Revision Conference of the International List of Causes of Death, held in Paris in 1900. The original classification, as indicated by its title, was used solely for coding causes of death, and did not provide a separate section for the mental disorders until the Fifth Revision of that List (1938). In that Revision, mental disorders were assigned only a single three-digit rubric with four subcategories within the section on Diseases of the Nervous System and Sense Organs: (a) mental deficiency; (b) schizophrenia; (c) manic depressive psychosis; and (d) all other mental disorders. The Conference for the Sixth Revision (1948) expanded the classification for use not only for causes of death but also for causes of morbidity. ICD-6 contained the first separate section on mental disorders (Section V). No major revisions were made in that section in ICD-7 (1955).

The international community of psychiatrists expressed considerable dissatisfaction with the classification of mental disorders in ICD-6 and ICD-7; consequently, it was not widely used (Stengel, 1959). As the importance of mental disorders as an international public health problem became more widely

Table 1

Distribution of the 3-Digit Categories of the ICD-9 by the Number of Digits Allocated to Each Category

<u>Major 3-digit categories</u>		<u>Digits allocated to category</u>	<u>Number of 3-digit categories</u>
I	Infectious and Parasitic Diseases	001-139	139
II	Neoplasms	140-239	100
III	Endocrine, Nutritional, and Metabolic Disorders	240-279	40
IV	Diseases of Blood and Blood-forming Organs	280-289	10
V	Mental Disorders	290-319	30
VI	Diseases of the Nervous System and Sense Organs	320-389	70
VII	Diseases of the Circulatory System	390-459	70
VIII	Diseases of the Respiratory System	460-519	60
IX	Diseases of the Digestive System	520-579	60
X	Diseases of the Genito-urinary System	580-629	50
XI	Complications of Pregnancy, Childbirth, and Puerperium	630-679	50
XII	Diseases of the Skin and Subcutaneous Tissue	680-709	30
XIII	Diseases of the Musculoskeletal System and Connective Tissue	710-739	30
XIV	Congenital Abnormalities	740-759	20
XV	Certain Conditions Originating in the Prenatal Period	760-779	20
XVI	Symptoms, Signs, and Ill-defined Conditions	780-799	20
XVII	Injury and Poisoning	800-999	200
		TOTAL	999

recognized, the need for an internationally acceptable classification of mental disorders became increasingly urgent. Accordingly, the World Health Organization developed an active program for revising the content and form of that classification to reflect new knowledge of the differential characteristics of specific mental disorders and their diagnosis and treatment to meet the increasing needs of health and social agencies, research workers, and users of health statistics for more detailed statistical and epidemiological data on mental disorders. The mental disorders chapter of ICD-8, adopted in 1965, reflected these changes (WHO, 1969).

To achieve more uniform usage of the terms in the mental disorders classification of ICD-8, WHO convened a working group of experts from dif-

ferent countries for the purpose of preparing a *Glossary of Mental Disorders and Guide to Their Classification* for use in conjunction with ICD-8 (WHO, 1974). The main aim of this glossary was:

to ensure as far as possible that those who apply it will arrive at a uniform use of the principal diagnostic terms current in psychiatry. In addition to helping to minimize the discrepancies among the diagnostic concepts used by psychiatrists in different countries for the statistical reporting of mental illness, use of the Glossary in publications dealing with either clinical work or research will also assist psychiatrists from different countries and schools of thought in understanding each other's work and concepts.

The Ninth Revision of ICD

To develop revision proposals for ICD-9, WHO initiated an intensive program to obtain information on problems encountered by psychiatrists in different countries in the use of the mental disorders section of ICD-8 and to formulate recommendations for their solutions (Shepherd, et al, 1968). This program resulted in the classification of mental disorders that appears in ICD-9, adopted by the World Health Assembly in 1975 (WHO, 1977).

As stated by WHO (1978):

Changes and new categories in the International Classification of Diseases (ICD-9) have been introduced only for sound reasons and after much consideration. As far as possible, the changes in Chapter V (ICD-9) have been based upon evidence that the new codes function better than the old ones. Some of this evidence, and a large proportion of other changes based upon discussion and consideration of different viewpoints, emanated from the World Health Organization's program on the standardization of psychiatric diagnosis, classification and statistics. A central feature of this program was a series of eight international seminars held annually between 1965 and 1972, each of which focused upon a recognized problem area in psychiatric diagnosis. Psychiatrists from more than 40 countries participated, and the documents and proposals that were used to produce the recommendations for ICD-9 in the eighth and final seminar were seen and commented upon by many more.

The first seven of the seminars focused on the classification of major groups of psychiatric disorders, and the last seminar on program review.

Dr. Jack Ewalt, past President of the American Psychiatric Association, and Dr. Henry Brill, former Chairman of the APA Task Force on Nomenclature and Statistics, played active roles in these seminars and in the development of the final classification of mental disorders of ICD-9. Other members of the Association who participated in and made important contributions to the recommendations developed in several of the seminars included: Dr. Leon Eisenberg (disorders of childhood) and Drs. George Tarjan, Julius Richmond, and J. Wortis (mental retardation) (WHO, 1970, 1971, 1972, 1973).

<u>Place</u>	<u>Year</u>	<u>Subject</u>
London	1965	Functional psychoses, with emphasis on schizophrenia
Oslo	1966	Borderline psychosis, reactive psychosis
Paris	1967	Psychiatric disorders of childhood
Moscow	1968	Mental disorders of old age
Washington, DC	1969	Mental retardation
Basle	1970	Neurotic and psychosomatic disorders
Tokyo	1971	Personality disorders and drug addiction
Geneva	1972	Summary, conclusions, recommendations, and proposals for further research

WHO Glossary of Mental Disorders

A major innovation of the mental disorders section of ICD-9 is the incorporation of a glossary as an integral part of that section. It is the only section of ICD-9 that contains such a glossary. The reason for this, as stated by WHO, is as follows (WHO, 1977a):

This section of the Classification differs from the others in that it includes a glossary, prepared after consultation with experts from many different countries, defining the contents of the rubrics. This difference is considered to be justified because of the special problems posed for psychiatrists by the relative lack of independent laboratory information upon which to base their diagnoses. The diagnosis of many of the most important mental disorders still relies largely upon descriptions of abnormal experience and behaviour, and without some guidance in the form of a glossary that can serve as a common frame of reference, psychiatric communications easily became unsatisfactory at both clinical and statistical levels.

Many well-known terms have different meanings in current use, and it is important for the user to use the glossary descriptions and not merely the category titles when searching for the best fit for the condition he is trying to code. This is particularly important if a separate national glossary also exists.

Differences between the Mental Disorders Sections of ICD-8 and ICD-9

Table 2 provides a comparison of the three-digit categories of mental disorders in ICD-8 and ICD-9.

Table 2
Comparison of ICD-8 and ICD-9 3-Digit Categories of Mental Disorders

ICD-8	ICD-9
PSYCHOSES (290-299)	ORGANIC PSYCHOTIC CONDITIONS (290-294)
290 Senile and presenile dementia	290 Senile and presenile organic psychotic conditions
291 Alcoholic psychosis	291 Alcoholic psychoses
292 Psychosis associated with intracranial infection	292 Drug psychoses
293 Psychosis associated with other cerebral condition	293 Transient organic psychotic conditions
294 Psychosis associated with other physical condition	294 Other organic psychotic conditions (chronic)
295 Schizophrenia	OTHER PSYCHOSES (295-299)
296 Affective psychoses	295 Schizophrenic psychoses
297 Paranoid states	296 Affective psychoses
298 Other psychoses	297 Paranoid states
299 Unspecified psychosis	298 Other nonorganic psychoses
	*299 Psychoses with origin specific to childhood
NEUROSES, PERSONALITY DISORDERS AND OTHER NONPSYCHOTIC MENTAL DISORDERS (300-309)	NEUROTIC DISORDERS, PERSONALITY DISORDERS AND OTHER NONPSYCHOTIC MENTAL DISORDERS (300-316)
300 Neuroses	300 Neurotic disorders
301 Personality disorders	301 Personality disorders
302 Sexual deviations	302 Sexual deviations and disorders
303 Alcoholism	303 Alcohol dependence syndrome
304 Drug dependence	304 Drug dependence
305 Physical disorders of presumably psychogenic origin	*305 Nondependent abuse of drugs
306 Special symptoms not elsewhere classified	306 Physiological malfunction arising from mental factors
307 Transient situational disturbances	307 Special symptoms or syndromes not elsewhere classified
308 Behavior disorders of childhood	*308 Acute reaction to stress
309 Mental disorders not specified as psychotic associated with physical conditions	*309 Adjustment reaction
	310 Specific nonpsychotic mental disorders following organic brain damage
	*311 Depressive disorder, not elsewhere classified
	*312 Disturbance of conduct not elsewhere classified
	*313 Disturbance of emotions specific to childhood and adolescence
	*314 Hyperkinetic syndrome of childhood
	*315 Specific delays in development
	*316 Psychic factors associated with diseases classified elsewhere
MENTAL RETARDATION (310-315)	MENTAL RETARDATION (317-319)
310 Borderline mental retardation	317 Mild mental retardation
311 Mild mental retardation	318 Other specified mental retardation
312 Moderate mental retardation	319 Unspecified mental retardation
313 Severe mental retardation	
314 Profound mental retardation	
315 Unspecified mental retardation	

Adapted from: World Health Organization (1978) *Mental Disorders: Glossary and Guide to Their Classification in Accordance with the Ninth Revision of the International Classification of Diseases*, Geneva, WHO, 1978.

* New categories in ICD-9 that were not in ICD-8

The classification of the following disorders was thoroughly recast in ICD-9:

Disorder	ICD-9 Codes
Affective psychoses	296
Organic mental disorders	290, 293, 294
Acute reaction to stress	308
Adjustment reaction	309
Specific nonpsychotic mental disorder following organic brain damage	310
Disturbance of conduct, not elsewhere classified	312
Disturbance of emotions specific to childhood and adolescence	313
Hyperkinetic syndrome of childhood	314
Specific delays in development	315
Alcohol disorders	291, 303, 305
Drug disorders	292, 304, 305

Several new three-digit categories were also added:

- 299 Psychoses with origin specific to childhood
- 305 Nondependent abuse of drugs
- 308 Acute reaction to stress
- 309 Adjustment reaction
- 311 Depressive disorder, not elsewhere classified
- 312 Disturbance of conduct, not elsewhere classified
- 313 Disturbance of emotions specific to childhood and adolescence
- 314 Hyperkinetic syndrome of childhood
- 315 Specific delays in development
- 316 Psychic factors associated with diseases classified elsewhere

Elimination of Combination Categories from ICD-9

ICD-9 also differs from ICD-8 in that it does not include so-called "combination categories," for coding combined mental and physical disorders such as organic mental disorders and mental retardation. To illustrate, in certain instances an ICD-8 category designated a mental disorder associated with a specific physical condition (e.g., psychosis with cerebral arteriosclerosis); in other instances, the category consisted of a specified mental condition and a general class of associated physical disorders (e.g., moderate mental retardation following infections and intoxications). "Combination categories" such as these have been eliminated from ICD-9 and replaced by categories that require coding on two independent axes. Thus, a psychotic condition arising from a physical disorder would be classified by using two code numbers: one for the mental disorder, and one for the underlying physical disorder. The following are categories in which multiple coding is necessary:

Disorder	ICD-9 Codes
Senile and presenile organic psychotic conditions	290
Transient organic psychotic conditions	293
Other organic psychotic conditions (chronic)	294
Specific nonpsychotic mental disorder following organic brain damage	310
Physiological malfunction arising from mental factors	306
Psychic factors associated with diseases classified elsewhere	316
Mental retardation	317 — 319

The use of the second code will require the clinician to familiarize himself or herself with all of the sections of the ICD and its alphabetical index (WHO, 1977a,b). This index assists in locating the code number for the associated condition.

Multiaxial Classification

The concept of a multiaxial classification system for mental disorders was first proposed at the WHO seminar on the mental disorders of childhood, in Paris in 1967 (Rutter et al., 1969). The condition of the child was to be recorded on three axes: clinical syndrome, intellectual level, and etiologic factors. Two years later, during the seminar in Washington on the problems of classification of mental retardation, another axis was suggested for coding associated social and cultural factors (Tarjan et al., 1972).

WHO has initiated a number of international studies to obtain empirical data about the usefulness of the multiaxial classification. In one of these, carried out in the United Kingdom and involving a large number of child psychiatrists, a series of patients were assessed using both the triaxial approach and the ICD. Case histories were also used to assess the agreement between psychiatrists using these two classificatory systems. The results of this study clearly demonstrated that a multiaxial classification system can be used, and that it provides more and better data about the patients seen and assessed (Rutter et al., 1975). This study has now been expanded, and psychiatrists from several European countries are participating. Similar studies are about to begin in other countries. Some of these are concerned with the classification of disorders in old age, and others with the classification of mental disorders in criminals.

Other Details of ICD-9

The interested reader will find more details on the development of the ICD-9 Classification of Mental Disorders and its content in *Mental Disorders: Glossary and Guide to Their Classification in accordance with the Ninth Revision of the ICD* (WHO, 1978), and a review of other highlights in a paper by Kramer et al. (1979).

International Classification of Diseases, Clinical Modification (ICD-9-CM)

As stated earlier, the ICD is primarily a classification of diseases for use in coding morbidity and mortality data for statistical purposes, and it has been adapted for use in clinical situations for the indexing of hospital records by disease and procedure (H-ICD-A). However, in the United States, clinicians and others responsible for the care of patients found they needed a classification with more specificity than that provided by ICD-9. Accordingly, the National Center for Health Statistics convened during 1977 a steering committee to advise the Council on Clinical Classifications on how to modify ICD-9 to satisfy this need. This Council was sponsored by the following organizations: American Academy of Pediatrics, American College of Obstetricians and Gynecologists, American College of Physicians, American College of Surgeons, American Psychiatric Association, and Commission on Professional and Hospital Activities. The task forces on classification of these organizations provided clinical guidance and technical input to the development of ICD-9-CM, the *Clinical Modification of the World Health Organization's International Classification of Diseases, 9th Revision*. As stated in the introduction to ICD-9-CM (1978):

The term 'clinical' is used to emphasize the modification's intent: to serve as a useful tool in the area of classification of morbidity data for indexing of medical records, medical care review, and ambulatory and other medical care programs, as well as for basic health statistics. To describe the clinical picture of the patient, the codes must be more precise than those needed only for statistical groupings and trends analysis.

ICD-9-CM is compatible with its parent system, ICD-9, thus meeting the need for comparability of morbidity and mortality statistics at the international level. This was accomplished by: (a) keeping the contents and the sequence of the three-digit rubrics of ICD-9 unchanged; (b) not adding new three-digit rubrics to the main body of the classification; (c) adding a fifth digit to the existing ICD-9 rubrics; and (d) creating a few four-digit codes in existing three-digit rubrics only when the necessary detail could not be accommodated by the use of a fifth-digit subclassification.

As of the time when WHO had essentially completed its work on ICD-9, the American Psychiatric Association's Task Force on Nomenclature and Statistics was still in the midst of preparing the third edition of its Diagnostic and Statistical Manual of Mental Disorders (DSM-III). As a result, it was not possible to have revised diagnostic terms submitted in time for inclusion in ICD-9. However, the Chair of this APA Task Force was invited by the Council on Clinical Classifications to submit for inclusion in ICD-9-CM, those DSM-III terms not included in ICD-9. After ICD-9-CM went into effect on 1 January 1979, further changes were made in the still evolving DSM-III classification. All of the terms in the final DSM-III classification are either included in the ICD-9-CM volume itself or published as addenda to the ICD-9-CM in issues of

Medical Record News as recommended terms or inclusion terms (acceptable as alternative terms).*

The classification of affective psychoses in ICD-9-CM (code 296) departs considerably from that in ICD-9. It is important for the users of this category to be aware of the fact that, in some instances, the same four-digit code numbers refer to different conditions in ICD-9 and ICD-9-CM. For example, the code 296.2 in ICD-9 is "Manic depressive psychoses, circular type but currently manic," whereas in ICD-9-CM, the same code is used for "Major Depressive Disorder, Single Episode."

It should also be noted that following the practice of ICD-9, "combination codes" are not included in ICD-9-CM. Therefore, it is important for clinicians and others using ICD-9-CM to become acquainted with codes for other diseases and other conditions that are to be entered on Axis III of the DSM-III classification (for Physical Disorders and Conditions).

Summary

The International Classification of Diseases (ICD) is an essential tool for the collection and dissemination of comparable mortality and morbidity data throughout the world. Mental disorders have been assigned an increasingly prominent place in the ICD; and the proposals for their classification in the 9th Revision, which became effective as of 1 January 1979, have been formulated on the basis of an extensive WHO program involving a series of seminars and consultations with leading mental health experts in many countries.

An accompanying glossary and guide to the classification rubrics for the mental disorders were developed for the first time in connection with the 8th Revision. A major innovation in the 9th Revision is the incorporation of the glossary within the text of the section on mental disorders.

The new elements in the ICD-9 section on mental disorders include thoroughly recast rubrics for several categories, including affective disorders and psychiatric conditions specific to childhood. The "combination categories" for coding associations between mental and physical disorders are eliminated and replaced by categories requiring independent coding.

The next revision of the ICD will have to take into account a variety of needs that have emerged since the publication of ICD-9. These include: multi-axial classification methods, classification of disabilities, adaptation of the ICD for use in primary health care, and standardization of medical nomenclature on multilingual bases.

A major development in the United States was the preparation of the ICD-9-CM (Clinical Modification) to provide the additional specificity required by clinicians, research workers, epidemiologists, program planners, medical record librarians, and administrators of inpatient, outpatient, and community programs.

The experiences derived from the use of ICD-9, ICD-9-CM, and DSM-III in the United States will be invaluable for those who will participate in the development of ICD-10.

* Mr. Robert Seeman, Chief Nosologist, Council on Clinical Classifications, provided invaluable consultation that helped achieve compatibility between the DSM-III and ICD-9-CM classifications.

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* A list of published material and reports is available from the World Health Organization upon request.

OFFPRINT OF MENTAL DISORDERS CHAPTER OF ICD-9

From Mental Disorders: Glossary and Guide to Their Classification in Accordance with the Ninth Revision of the International Classification of Diseases, World Health Organization, Geneva, 1978

This section of the Classification differs from the others in that it includes a glossary, prepared after consultation with experts from many different countries, defining the content of the rubrics. This difference is considered to be justified because of the special problems posed for psychiatrists by the relative lack of independent laboratory information upon which to base their diagnoses. The diagnosis of many of the most important mental disorders still relies largely upon descriptions of abnormal experience and behaviour, and without some guidance in the form of a glossary that can serve as a common frame of reference, psychiatric communications easily become unsatisfactory at both clinical and statistical levels.

Many well-known terms have different meanings in current use, and it is important for the user to use the glossary descriptions and not merely the category titles when searching for the best fit for the condition he is trying to code. This is particularly important if a separate national glossary also exists.

The instructions "Use additional code to identify . . ." are important because of the nature of many psychiatric conditions in which two or more codes are necessary to describe the condition and the associated or causal factors. It should be used whenever possible.

In cases where no other information is available except that a mental disorder is present, the code V40.9 (unspecified mental or behavioural problems) can be used.

PSYCHOSES (290-299)

Mental disorders in which impairment of mental function has developed to a degree that interferes grossly with insight, ability to meet some ordinary demands of life or to maintain adequate contact with reality. It is not an exact or well defined term. Mental retardation is excluded.

ORGANIC PSYCHOTIC CONDITIONS (290-294)

Syndromes in which there is impairment of orientation, memory, comprehension, calculation, learning capacity and judgement. These are the essential features but there may also be shallowness or lability of affect, or a more persistent disturbance of mood, lowering of ethical standards and exaggeration or emergence of personality traits, and diminished capacity for independent decision.

Psychoses of the types classifiable to 295-298 and without the above features are excluded even though they may be associated with organic conditions.

The term '*dementia*' in this glossary includes organic psychoses as just specified, of a chronic or progressive nature, which if untreated are usually irreversible and terminal.

The term '*delirium*' in this glossary includes organic psychoses with a short course in which the above features are overshadowed by clouded consciousness, confusion, disorientation, delusions, illusions and often vivid hallucinations.

Includes: psychotic organic brain syndrome

Excludes: nonpsychotic syndromes of organic aetiology (see 310.-)
 psychoses classifiable to 295-298 and without the above features but associated with physical disease, injury or condition affecting the brain [e.g., following childbirth]; code to 295-298 and use additional code to identify the associated physical condition

290 Senile and presenile organic psychotic conditions

Excludes: psychoses classifiable to 295-298.8 occurring in the senium without dementia or delirium (295-298)
 transient organic psychotic conditions (293.-)
 dementia not classified as senile, presenile, or arteriosclerotic (294.1)

290.0 *Senile dementia, simple type*

Dementia occurring usually after the age of 65 in which any cerebral pathology other than that of senile atrophic change can be reasonably excluded.

Excludes: mild memory disturbances, not amounting to dementia, associated with senile brain disease (310.1)
 senile dementia:
 depressed or paranoid type (290.2)
 with confusion and/or delirium (290.3)

290.1 *Presenile dementia*

Dementia occurring usually before the age of 65 in patients with the relatively rare forms of diffuse or lobar cerebral atrophy. Use additional code to identify the associated neurological condition.

Brain syndrome with presenile brain disease

Circumscribed atrophy of the brain

Dementia in:

 Alzheimer's disease

 Pick's disease of the brain

Excludes: arteriosclerotic dementia (290.4)
 dementia associated with other cerebral conditions (294.1)

290.2 *Senile dementia, depressed or paranoid type*

A type of senile dementia characterized by development in advanced old age, progressive in nature, in which a variety of delusions and hallucinations of a persecutory, depressive and somatic content are also present. Disturbance of the sleep/waking cycle and preoccupation with dead people are often particularly prominent.

Senile psychosis NOS

Excludes: senile dementia:
 with confusion and/or delirium (290.3)
 NOS (290.0)

290.3 *Senile dementia with acute confusional state*

Senile dementia with a superimposed reversible episode of acute confusional state

Excludes: senile:

dementia NOS (290.0)

psychosis NOS (290.2)

290.4 *Arteriosclerotic dementia*

Dementia attributable, because of physical signs [on examination of the central nervous system] to degenerative arterial disease of the brain. Symptoms suggesting a focal lesion in the brain are common. There may be a fluctuating or patchy intellectual defect with insight, and an intermittent course is common. Clinical differentiation from senile or presenile dementia, which may coexist with it, may be very difficult or impossible. Use additional code to identify cerebral atherosclerosis (437.0).

Excludes: suspected cases with no clear evidence of arteriosclerosis (290.9)

290.8 *Other***290.9 *Unspecified*****291 *Alcoholic psychoses***

Organic psychotic states due mainly to excessive consumption of alcohol; defects of nutrition are thought to play an important role. In some of these states, withdrawal of alcohol can be of aetiological significance.

Excludes: alcoholism without psychosis (303)

291.0 *Delirium tremens*

Acute or subacute organic psychotic states in alcoholics, characterized by clouded consciousness, disorientation, fear, illusions, delusions, hallucinations of any kind, notably visual and tactile, and restlessness, tremor and sometimes fever.

Alcoholic delirium

291.1 *Korsakov's psychosis, alcoholic*

A syndrome of prominent and lasting reduction of memory span, including striking loss of recent memory, disordered time appreciation and confabulation, occurring in alcoholics as the sequel to an acute alcoholic psychosis [especially delirium tremens] or, more rarely, in the course of chronic alcoholism. It is usually accompanied by peripheral neuritis and may be associated with Wernicke's encephalopathy.

Alcoholic polyneuritic psychosis

Excludes: Korsakov's psychosis:

NOS (294.0)

nonalcoholic (294.0)